

ENCOUNTER 2 OF 5

Date of Service: 2024-08-10 | Type: OUTPATIENT VISIT | Specialty: Neurology | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Marcus Webb, MD	Location:	New Amandaville Neurology Associates
---------------------	---------------------	-----------	--------------------------------------

Chief Complaint

Follow-up evaluation of persistent left foot/toe numbness and tingling despite physical therapy.

Interval History

Ms. Gardner returns for neurology follow-up approximately eight weeks after her initial orthopedic evaluation. She completed a 4-week course of physical therapy targeting gait mechanics and nerve gliding exercises, which she reports provided partial but incomplete relief of her symptoms. Numbness and tingling in the left toes persist, now rated 3/10 to 4/10 at its worst, still exacerbated by prolonged standing and walking, and relieved somewhat with rest and elevation. She continues to deny bilateral symptoms, bowel or bladder dysfunction, or new weakness. She denies any interval trauma.

Laboratory results obtained after the initial visit, including CBC, CMP, hemoglobin A1c, and TSH, returned within normal limits, arguing against a systemic metabolic driver such as diabetic peripheral neuropathy or thyroid-related neuropathy. Weight-bearing X-rays of the left foot obtained at the initial visit likewise showed no acute osseous abnormality, fracture, or significant degenerative change. Given the persistence of focal sensory symptoms despite a benign metabolic and radiographic work-up, she was referred to neurology for further characterization, including consideration of electrodiagnostic testing and advanced imaging to evaluate for a focal compressive lesion such as an intraneural ganglion cyst or peripheral nerve entrapment.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean

section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	124/70 mmHg	Heart Rate	70 bpm
Temperature	97.9 F	Respiratory Rate	16 breaths/min
Pain Score	3/10	SpO2	98% RA
Weight	154 lbs	Height	5 ft 5 in
BMI	25.6 kg/m2		

Prior Diagnostic Results Reviewed

Test	Result	Reference Range	Flag
CBC (WBC, Hgb, Hct, Plt)	All within normal limits	See panel	Normal
CMP (glucose, electrolytes, renal, hepatic)	All within normal limits	See panel	Normal
Hemoglobin A1c	5.3 %	< 5.7 %	Normal
TSH	2.1 uIU/mL	0.4 - 4.0 uIU/mL	Normal
X-ray, Left Foot (3 views)	No acute osseous abnormality	N/A	Normal

Physical Examination

General

Alert, well-appearing female in no acute distress.

Neurological Examination

Cranial nerves II-XII grossly intact. Motor strength 5/5 throughout bilateral upper and lower extremities, including tibialis anterior, gastrocnemius, and extensor hallucis longus. Sensation to light touch and pinprick decreased over the dorsal and plantar aspect of the left first through third toes, with a positive Tinel's sign over the medial ankle at the tarsal tunnel. Sensation intact and symmetric elsewhere. Deep tendon reflexes 2+ and symmetric at patella and Achilles bilaterally. No clonus, no Babinski sign. Gait is narrow-based and non-antalgic; able to heel walk and toe walk without difficulty. Romberg test negative.

Musculoskeletal

Mild fullness palpated along the course of the posterior tibial nerve near the tarsal tunnel; no overt mass definitively palpable. No joint effusion. Left knee with mild stiffness, full range of motion.

Diagnostic Studies Ordered / Reviewed

Test	Result	Reference Range	Flag
Nerve Conduction Study (NCS), left lower extremity	Mild focal slowing, posterior tibial nerve	Normal conduction velocity	Abnormal
Electromyography (EMG), left lower extremity	No active denervation	No spontaneous activity	Normal
MRI, Left Foot/Ankle (ordered this visit)	Pending	N/A	-

Nerve conduction studies performed today demonstrate mild focal slowing of the posterior tibial nerve consistent with a focal compressive process at or near the tarsal tunnel, without evidence of a more diffuse peripheral polyneuropathy. Electromyography of the left distal lower extremity muscles shows no evidence of active denervation at this time.

Assessment

1. Focal left tarsal tunnel neuropathy, electrodiagnostically confirmed, most consistent with a localized compressive lesion (e.g., ganglion cyst, tenosynovial cyst) versus structural entrapment; systemic/metabolic causes have been reasonably excluded.
2. Hypertension, controlled.

3. Mild intermittent asthma, controlled.
4. Primary hypothyroidism, controlled.

Plan

1. Order MRI of the left foot/ankle with and without contrast to evaluate for a structural or compressive lesion along the posterior tibial nerve/tarsal tunnel.
2. Refer to orthopedic surgery (foot and ankle) for surgical consultation pending MRI results, given electrodiagnostic evidence of a focal compressive neuropathy.
3. Continue current physical therapy home exercise program.
4. Continue home medications without change.
5. Trial of gabapentin 100 mg PO at bedtime was discussed; patient elected to defer pharmacologic therapy for now, preferring to proceed to imaging and surgical consultation first.
6. Follow up with neurology after MRI and surgical consultation, or sooner if symptoms progress.

Patient Education / Instructions

Findings and their implications were reviewed in detail with the patient, including the likely focal, structural nature of her nerve compression and the rationale for proceeding to MRI and surgical referral. She was counseled on warning signs including rapidly progressive weakness, foot drop, or new bilateral symptoms that would warrant urgent re-evaluation. She verbalized understanding and agreed with the proposed plan.

Care Team

Name	Role	Contact
Marcus Webb, MD	Attending Neurologist	555-201-7710
Priya Chandrasekaran, RN	Neurology Clinic Nurse	555-201-7715
EMG/NCS Technician: Rob Alvarez, R.NCS.T.	Electrodiagnostic Technologist	555-201-7720
Dr. Anita Reyes, MD	Orthopedic Surgery (co-managing)	555-201-4410

Electrodiagnostic Study Detail

Test	Result	Reference Range	Flag
Posterior Tibial Motor NCV (left)	38 m/s	> 41 m/s	Mildly Abnormal
Posterior Tibial Distal Latency (left)	6.8 ms	< 6.0 ms	Mildly Prolonged
Sural Sensory NCV (left)	44 m/s	> 40 m/s	Normal
Tibialis Anterior EMG (left)	No spontaneous activity, normal recruitment	Normal pattern	Normal
Abductor Hallucis EMG (left)	No spontaneous activity, normal recruitment	Normal pattern	Normal

The electrodiagnostic pattern of an isolated, mild prolongation of posterior tibial distal motor latency with reduced conduction velocity localized to the tarsal tunnel segment, in the setting of a normal sural sensory response and no denervation on needle EMG, is most consistent with an early, focal demyelinating conduction abnormality of the posterior tibial nerve at the tarsal tunnel rather than a generalized peripheral polyneuropathy.

Care Coordination Note

MRI order for the left foot/ankle with and without contrast was submitted for prior authorization through the patient's Kaiser plan; the patient was advised that scheduling would be arranged by the imaging center within 1-2 weeks pending authorization. A referral to orthopedic surgery (foot and ankle subspecialty, continuing with Dr. Reyes given prior relationship) was placed for surgical consultation once imaging is finalized. The patient was given educational materials regarding tarsal tunnel syndrome and what to expect from MRI. Copies of today's note and electrodiagnostic report were sent to Dr. Reyes and to the patient's primary care physician, Dr. Susan Whitfield, for continuity of care.

Electronically signed by: **Marcus Webb, MD**
Department: Neurology
Signed: 2024-08-10 2:15 PM